

MEMO

To: VP of Population Health
From: Ameer Salameh, Analyst
Date: October 2026
Re: Where to put next year's care management and pharmacy dollars

Recommendation

Fund pharmacy outreach for the 270 members who are just under the adherence threshold. In the base case it saves about \$53,000 a year, roughly \$5 for every \$1 spent. Do not fund generic care management for last year's highest spenders: the base case loses about \$224,000. Send ESRD members to a kidney care approach instead, and pilot transitional care after discharge.

What the claims show (2022, 8,175 members)

- **Spend is rising and concentrated.** Medical spend is \$1,665 per member per month, up 39% since 2015. The top 5% of members account for 47.5% of it.
- **ESRD is the cost that persists.** Members with end-stage renal disease are 10% of member months and 32% of spend (\$5,447 PMPM). Outpatient dialysis is the fastest growing category.
- **Most other high spenders get cheaper on their own.** The 2021 top 5% who don't have ESRD cost 39% less in 2022 with no program. A before and after comparison would credit that drop to whatever program we ran.
- **Adherence gaps are large.** 81.6% of statin users are adherent (PDC 80% or more), but only 20.5% on blood pressure (RAS) drugs and 14.3% on diabetes drugs. 270 members sit between 60% and 79%.
- **Readmissions are climbing.** 10.5% in 2022, up from 2.8% in 2015. Still below the 16.9% national Medicare rate.

The two options, priced

Program	People	Yearly cost (base)	Net savings: low / base / high	Call
Pharmacy outreach to adherence near misses	270	\$13,500	-\$16K / \$53K / \$261K	Fund
Care management for 2021 top 5%, non-ESRD	167	\$328,656	-\$541K / -\$224K / \$315K	Don't fund as designed

Care management only breaks even if it cuts admissions 25%. The best of 15 programs in Medicare's care coordination trials cut them 17%, and none produced net savings (Peikes et al., JAMA 2009). Outreach savings use published estimates (Roebuck et al., Health Affairs 2011) at half value, since that study is observational.

Next steps

1. Start outreach in Q1 using the worklist, closest to 80% first. Hold back a random 20% as a comparison group and track PDC monthly.
2. Work with dialysis providers and nephrology on the ESRD group. MSSP benchmarks ESRD separately, so track it separately too.
3. Pilot transitional care for discharges: a call within 48 hours and a visit within 7 days, measured against a comparison group.
4. Rerun this on our real claims before committing money. The build, QA checks, and ROI model are ready to point at new data.

Caveat: this analysis uses CMS synthetic Medicare data. The methods are real; the people are not. Full SQL, QA log, and assumptions are in the project repo.